

Experience with the panel survey approach to establishing MIDs

To establish best estimates of MIDS, the WHO has conducted panel surveys for their COVID-19 guidelines ¹ for a number of outcomes and subsequently applied these to their influenza guidelines ². BMJ Rapid

Recommendations regarding PCSK9 inhibitors and ezetimibe in patients with risk of cardiovascular events have also used the approach. ³ The Table presents their results. Although others might wish to consider these estimates, optimal MIDs are likely to be highly contextual.

A qualitative study has demonstrated the usefulness of the approach.⁴ The study involved individual interviews with 25 panelists from a WHO panel addressing COVID-19 and three BMJ Rapid Recommendation panels. Most panelists found the survey easy to follow and facilitated the incorporation of patient values and preferences in the tradeoffs between benefits and harms or burdens. The variation of patient preferences and uncertainty regarding patient values and preferences reflected in the surveys helped the panels decide on the strength of recommendations.

Table: MIDs generated using the survey approach

Condition	Outcome	MID
COVID-19 non-severe	Probability of hospitalization	Risk difference 15 per 1,000
COVID-19 non-severe	Duration of symptoms	Reduction by 1 day
COVID-19 all severity	Mortality	Risk difference 3 per 1,000
COVID-19 all severity	Long-COVID	Risk difference 60 per 1,000
Patients at elevated risk of cardiovascular events	Mortality	Risk difference 8 per 1,000
Patients at elevated risk of cardiovascular events	Non-fatal stroke	Risk difference 10 per 1,000
Patients at elevated risk of cardiovascular events	Non-fatal myocardial infarction	Risk difference 12 per 1,000

References

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